

INDICATIONS FOR SCREENING

Shared decision-making is recommended to determine when to screen; age, life expectancy, and risk factors, such as family history and race, should be taken into account.

Note: PSA screening is favored by the American Cancer Society, the American Urological Association, the National Comprehensive Cancer Network, and the U.S. Preventive Services Task Force. For specific societal recommendations, refer to the [ARUP Consult Prostate Cancer](#) topic.

ORDER

Total PSA

AND

Consider DRE as complementary baseline test

PSA <1 ng/mL

DRE normal
(if performed)Repeat testing
at 2- to 4-yr
intervalsDRE abnormal
(if performed)

PSA 1-3 ng/mL

DRE normal
(if performed)Repeat testing
at 1- to 2-yr
intervalsDRE abnormal
(if performed)Perform workup for
benign disease and
repeat testingPSA >3 ng/mL
OR
Rapid increase in PSA
OR
Suspicious DRE

PSA ≤3 ng/mL

PSA >3 ng/mL

DRE
(if not performed during initial
assessment)

Normal

Repeat testing
in 6-12 mos

Abnormal

Consider further testing
(%fPSA, 4Kscore, PHI)
to reduce need for
biopsyUrology consultation
and biopsy**Abbreviations**

%fPSA	Percent free PSA
DRE	Digital rectal exam
PHI	Prostate Health Index
PSA	Prostate-specific antigen

Reference

National Comprehensive Cancer Network. [NCCN Clinical Practice Guidelines in Oncology: prostate cancer early detection](#). Version 2.2025. Updated: Jun 2025; accessed: Aug 2025.